

ATTACHMENT 2

Applicant Project Description

Sutter Emeryville Medical Center
Project Description
Jun 8, 2026

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A. Overview.

The Emeryville Medical Center (EMC) Project is part of the Emery Yards (aka Emeryville Center of Innovation) campus purchased by Sutter Bay Hospitals, dba Alta Bates Summit Medical Center and Sutter Bay Medical Foundation, dba Sutter East Bay Medical Foundation (collectively, “Sutter”) from affiliates of Biomed Realty (“BMR”), which is a portion of what was formerly called the Chiron Development Plan campus. The original Chiron Development Plan site is located west of Hollis Street, south of Stanford Avenue, north of 45th street and east of the railroad tracks.

The City approved a Preliminary Development Plan (PDP) for the Chiron campus in 1995. Sutter acquired the portions of the PDP site that were owned by BMR in February 2025. Sutter proposes amendments to the PDP and two previously approved Final Development Plans (FDPs) for the development of a new hospital facility and associated helipad (Building 8); conversion of two existing research and development/office buildings (Buildings 2/3 and 4) into medical office and ambulatory care facilities and the construction of a new ambulatory care building (Building 1). The hospital would comprise a single building on a parcel previously approved for two office/research and development buildings (Buildings 8A and 8B) that have not been constructed. The PDP and FDP amendments would permit the proposed uses and include site plan revisions reflecting the design of the two proposed new buildings.

B. Project History.

The City approved the PDP for the Chiron campus in 1995. The PDP project was studied in the Chiron Development Plan Project EIR. Both the PDP and the EIR contemplate development of a maximum amount of square footage subject to certain design concepts. Though neither the PDP nor the EIR restricted the exact location, square footage or configuration of any building or use, both documents include illustrative diagrams demonstrating how the project might have been built out at various stages. The illustrative plan for full buildout of the PDP campus that appears in both the PDP and the EIR is shown in Figure 1 below, with the properties later acquired by Sutter marked with red dashed lines.



FIGURE 1: PDP ILLUSTRATIVE PLAN AT FULL BUILDOUT
WITH SUTTER-ACQUIRED PROPERTIES MARKED

The PDP identified then-existing buildings by letters and identifies the new buildings in its illustrative layouts by numbers. The PDP contemplated demolition of most then-existing buildings, and construction of Buildings 1 through 14, plus associated parking facilities. The PDP proposed to retain three buildings that were existing in 1995: (1) Building D, (2) the Process Development Unit (PDU) building, and (3) the Commercial Manufacturing Facility (CMF) building. Total development, including both existing and new construction but excluding parking structures, was proposed at 2,200,000 square feet. This buildout represented a proposed net increase of 1,588,000 square feet over the 612,000 square feet that existed in 1995.¹

The PDP allowed the developer the option either to turn Horton Street into a private access drive and replace it with a new Horton Bypass that would have extended outside the western boundary of the PDP site along the railroad tracks (see Figure 1), or to maintain Horton Street as a public street and extend it to connect with Stanford Avenue. The developer elected to proceed with the latter option and extended Horton Street to connect to Stanford Avenue. The

¹ 1995 Chiron Draft EIR, Table II.2, p. II.15. All square footage numbers are stated in terms of “usable space,” which excludes parking structures.

segment of Horton extending from 53rd street to Stanford Avenue has since been improved with bicycle lanes that are separated from the vehicle travel lanes by bollards.

Since approval of the PDP, Building 4, a laboratory building, was built at Hollis and 53rd Street pursuant to an FDP approved on August 20, 1996. A parking structure, Building 12A, was built on Horton Street near Stanford Avenue. A central utility plant was constructed as Building 7A on Chiron Way. Hollis Green Park, and Building 5 is no longer proposed to be built.² FDPs were approved by the City Council in February 2002 for Building 3, a research and development building north of Building 4, and for Building 7B, an expansion of Building 7A. However, neither of these buildings were constructed. Most recently, on February 25, 2021, the Planning Commission approved an FDP (by Resolution FDP19-002) for the construction of Buildings 1, 2/3, 8A, 8B, and 13/14, and the demolition of Building 7A and two other existing buildings. The City's Community Development Director administratively modified the conditions of approval to the 2021 FDP on June 9, 2023.

In 2006, the multinational Swiss pharmaceutical firm Novartis acquired Chiron. In November 2013, the diagnostics unit of Novartis was sold to Grifols, a multinational pharmaceutical and chemical manufacturer. The sites of diagnostic facilities were accordingly transferred to Grifols. As a result, the sites of Buildings 1, 2, 3, 4, 5, 7A, 8, 13, 14 and the PDU were retained by Novartis, while the sites of Buildings 6, 7B, 9, 10, 11, 12A, 12B the CMU and Building D were acquired by Grifols. BMR acquired all of the Novartis properties in March 2019, and Sutter acquired them in February 2025. The remainder of the PDP site continues to be owned by Grifols. The Sutter-owned properties are marked with red dashed lines in Figure 1 above; the unmarked properties belong to Grifols. Building 7A has since been demolished.

Grifols declined to assume obligations under the Chiron Development Agreement, allowing the Development Agreement to lapse on the Grifols-owned properties. Although Sutter assumed the obligations and rights under the Development Agreement, it expired on September 13, 2025.

² The PDP contemplated development of Building 5, which extends outside the PDP site to the "PG&E Tank Farm site." The PDP states on page 2-1: "The PG&E Tank Farm site is not part of the PUD Area but may be developed with the Life Sciences Center Project if Chiron is able to acquire the PG&E Tank Farm." BMR and subsequently Sutter did not acquire the PG&E Tank Farm site, and according to the Project Description for the 2020 FDP, Novartis did not indicate to BMR that it or Chiron had ever acquired that site.

C. Emeryville Medical Center (EMC) Project Site.

The EMC Project site comprises approximately 12 acres of the 22-acre PDP site. The EMC Project site includes the sites of buildings depicted in the PDP illustrative plan as proposed Buildings 1, 2, 3, 4, 8, 13, and 14, the PDU Building, and Hollis Green Park. All of the above currently exist, except for Buildings 1 and 8. Buildings 2 and 3 have been constructed as one single building (Building 2/3), as have Buildings 13 and 14 (Building 13/14).

Sutter Bay Hospitals owns the site previously proposed for Buildings 8A and 8B (now proposed as Building 8 for the hospital), and Sutter Bay Medical Foundation owns the sites of Buildings 1, 2/3, 4, and 13/14, and the PDU Building.

The current condition of all the Sutter-owned properties is shown in Figure 2, while the proposed plan is shown in Figure 3 below.

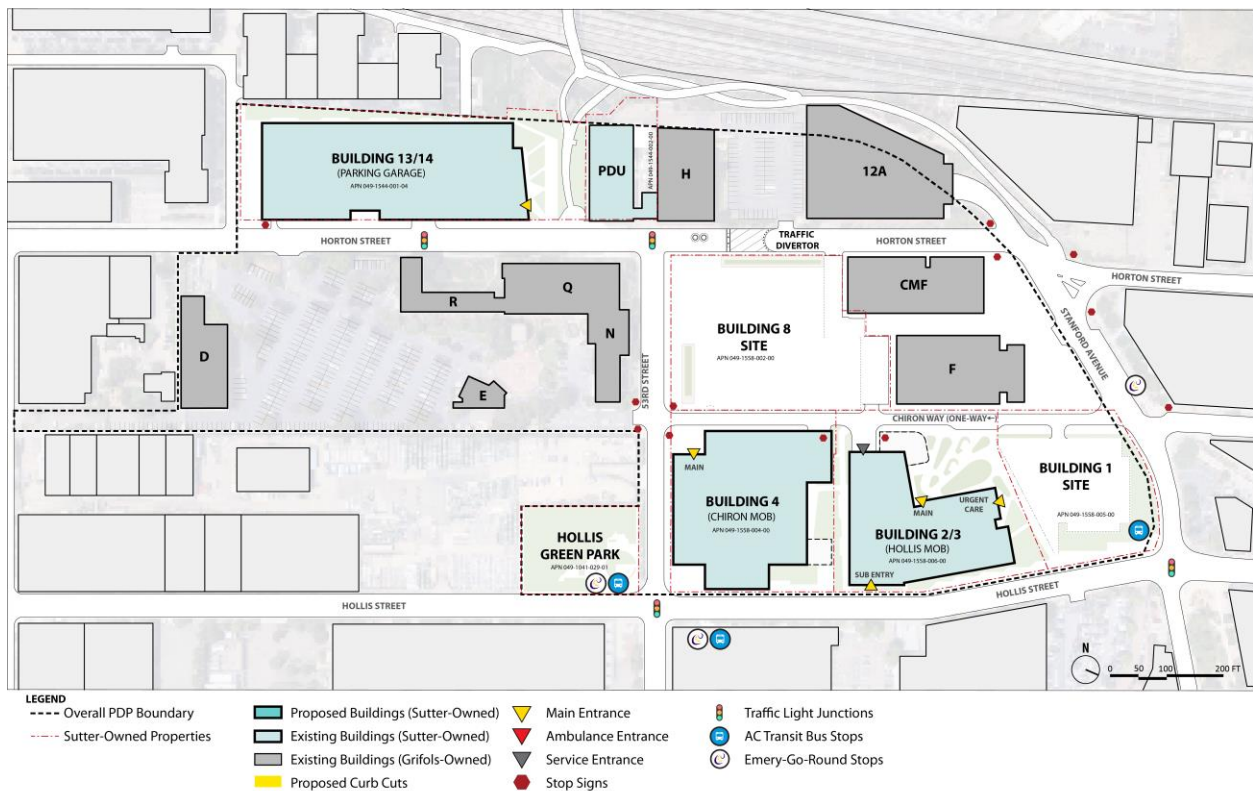


FIGURE 2: SUTTER-OWNED PROPERTIES EXISTING CONDITIONS

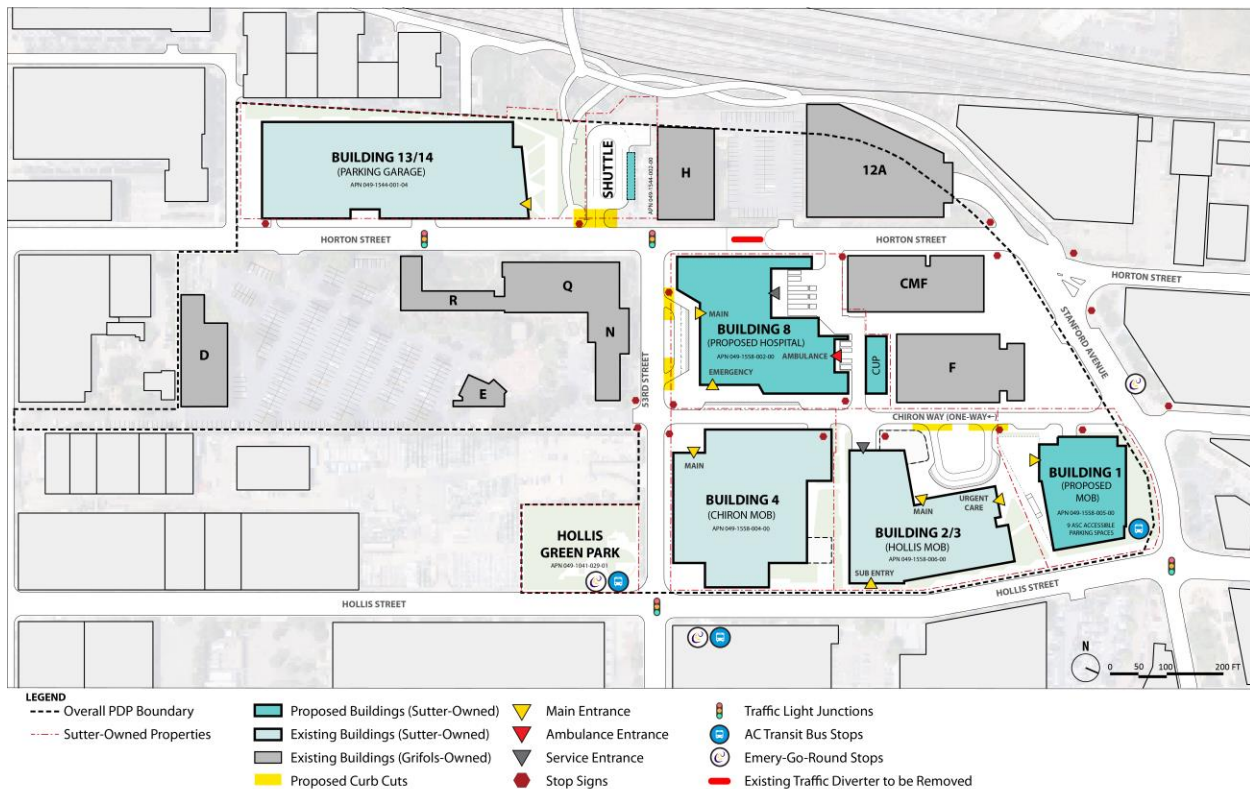


FIGURE 3: PROPOSED EMERYVILLE MEDICAL CENTER PROJECT SITE AND BUILDINGS

As shown in Figure 2, within the EMC Project site, the existing Sutter-Owned properties include Building 2/3, Building 4, Building 13/14, the PDU Building and Hollis Green Park.

Building Site 1 in the northeastern portion of the PDP site (at the southwest corner of Stanford Avenue and Hollis Street) is currently developed with surface parking that serves the existing Buildings 2/3 and 4. This area is surrounded by Building 2/3 to the south, Grocery Outlet corporate offices and surface parking across Hollis Street to the east, a sports bar and small office building across Stanford Avenue to the north, and the existing Building F within the PDP site immediately to the west of Chiron Way.

Building Sites 2 and 3 are located directly north of Building Site 4. Building Sites 2, 3, and 4 are currently developed with existing Buildings 2/3 (approximately 261,300 square feet) and 4 (approximately 292,500 square feet). This area is surrounded by Hollis Green Park and PG&E warehouse the south, Grocery Outlet corporate offices and surface parking across Hollis Street to the east, and the sites of the existing Building F and proposed Building 8 within the PDP site immediately to the west of Chiron Way.

Building Site 8, where the new hospital is proposed, is currently fenced and surrounded by Grifols properties to the north, west and south. This area is surrounded entirely by existing buildings within the PDP site: Building N to the south, Building H to the west, existing Building CMF and F to the north, and existing Building 4 to the east.

Building Sites 13 and 14, in the southwestern portion of the PDP site, are currently developed with Building 13/14, a structured parking garage. Immediately north of Building Sites 13 and 14 is the planned location for the City's Horton Landing Park connecting to the South Bayfront Bicycle & Pedestrian Bridge. To the east across Horton Street are existing Grifols buildings. To the west and south of the 13/14 site, is the Sherwin-Williams Mixed Use Project.

The square footages of existing buildings are estimated as follows:

Owner and Building	Square Footage
Sutter – 2/3	Approximately 261,300.
Sutter – 4	Approximately 292,500.
Sutter – 13/14	Parking Garage & an approximately 1,200 café
Sutter – PDU	Approximately 20,700
Grifols – CMF	85,000 estimated.
Grifols – D	15,000 estimated.
Grifols – F	27,400 according to the 2021 FDP Submission
Grifols – H, E, N, Q and R	157,300 according to the 2021 FDP Submission
Grifols - 12A	Parking Garage

The project site is within a Transit Priority Area. It is located within 0.5 miles of the Emeryville Amtrak station, which is considered a major transit stop, and San Pablo Avenue, which is considered a high-quality transit corridor because it is served by AC transit bus routes (72, 72M) that have service intervals of 15 minutes. The Emeryville Amtrak rail station provides access to the Capitol Corridor rail line that connects Sacramento in the north to San Jose in the south. Transit service to local destinations is provided by AC Transit and Emery Go Round bus services via bus stops located adjacent to the site along Hollis Street and Stanford Avenue. The Emery Go Round bus currently provides service with 15-minute headways at the Hollis Street and 53rd Street bus stop.

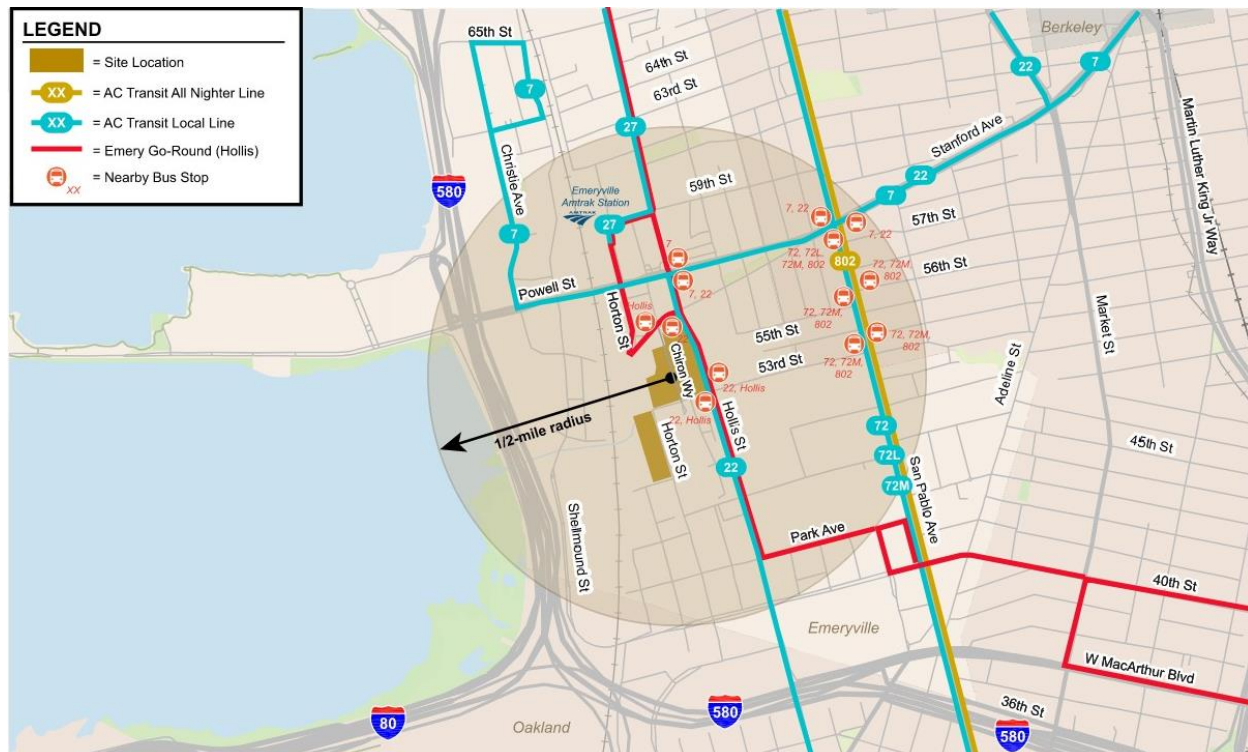


FIGURE 4: TRANSIT ROUTES AT PROJECT SITE

D. The Emeryville Medical Center (EMC) Project Design.

1. Overview of EMC Project.

The EMC Project proposes one new approximately 785,000 square feet hospital building (Building 8) and one new approximately 55,600 square feet (excluding parking) ambulatory care building (Building 1).

The EMC Project would also include tenant improvements to convert existing Buildings 2/3 and 4 to medical office and ambulatory care facilities. The existing Building 13/14 parking garage would support all buildings within the EMC Project - new Buildings 1 and 8 and the existing Buildings 2/3 and 4.

Building 1 would complete the build-out of the Hollis Street frontage between 53rd Street and Stanford Avenue. The proposed design for this street frontage would incorporate a view and access corridor between existing Building 2/3 and proposed Building 1. The opening between buildings would allow public access and views to the courtyard/open space that would be created along Chiron Way. Building 8 would complete the interior of the EMC Project site.

Building 13/14 would continue to operate as a garage that accommodates 1,991 vehicles as well as long-term and short-term bicycle parking, a café and a bicycle repair kiosk. Including grade and roof levels, the garage has 5 levels on the western side facing the Sherwin-Williams Mixed Use Project, stepping up to 11 levels on its eastern side facing Horton Street.

The approximate gross square footage and number of floors in each of the buildings on the EMC Project site after buildout of the EMC Project is shown in the following table. The two new buildings (B1 & B8) to be constructed as part of the EMC Project are in bold text and shaded:

Emeryville Medical Center Square Footage of Buildings at Buildout					
	B1	B2/3	B4	B8	B13/14
Total (Approx.)	55,600 (w/o parking)	270,700	292,500	785,000	1,200
Estimated Total for Buildings B1, B2/3, B4, B8 and B13/14: 1,405,000 square feet					

The PDP is proposed to be amended to allow the proposed hospital and ambulatory care/medical office uses and would also continue to allow administrative office uses. Some space would be used for loading docks, utility space and pedestrian links and connections within the buildings. Retail uses provided on the ground floor include the 1,200 square feet of existing publicly accessible cafe at the parking garage (Building 13/14), directly in front of Horton Plaza. There would be no retail spaces on the ground floor for the hospital (Building 8), but it would have approximately 500 square feet of gift shop and approximately 2,000 square feet of dining space on Level 02. There would be no retail space within Building 1, Building 2/3 and Building 4.

The total gross square footage proposed for the EMC Project site after buildout of the EMC Project would be approximately 1,405,000 gross square feet.

The net new population projected for the EMC Project is anticipated to be approximately 429 net new physicians, 2,981 net new employees and 12,060 net new patients and visitors for the Medical Office Buildings (MOBs) and the hospital compared to today. These estimates assume 0.5 visitors per patient for the MOBs and 1 visitor per bed for the hospital.

The square footage of all buildings in the EMC Project site would not cause the PDP site to exceed the 3.0 Floor Area Ratio (FAR) under the General Plan or the 2.33 FAR, in accordance with PDP Section 2.4.2. Upon buildout of the EMC Project, the entire PDP site, including the Grifols properties, would have approximately 1,689,700 gross square feet. After buildout of the EMC Project, the Sutter-owned properties would have approximately 1,405,000 square feet, compared to the 1,370,000 square feet depicted on these properties in the PDP's illustrative plan.

The following sections provide more detail on each component of the EMC Project.

2. Building 1.

Building 1 would consist of a 3 story, approximately 55,600 square feet Medical Office Building with a total building height of 50 feet tall (excluding rooftop penthouse structures). The ground floor would have an enclosed lobby area, with elevators up to the medical office, 53 parking spaces and other ancillary spaces. 44 parking spaces would be for patient use, while 9 accessible parking spaces specifically provided for the Ambulatory Surgery Center located within the adjacent Building 2/3. Levels 2 and 3 would accommodate ambulatory care and the building roof areas would be used for mechanical equipment and elevator penthouses. Building 1 is anticipated to have 25 physicians, 97 full time daily employees and a daily patient/visitor population of 1,088 people. The building would operate from 7:00 AM to 7:00 PM Mondays through Fridays, from 8:00 AM to 12:00 PM on Saturdays, and would remain closed on Sundays. Please see the Plan Set for all figures and plans indicating Building 1 as a future Medical Office Building.

Alternatively, the site could also be developed for residential use. Under this housing variant, it could accommodate a 50 feet tall (excluding rooftop penthouse structures) residential building for employees, totaling approximately 55,600 gross square feet. The ground floor would have an enclosed lobby area, with elevators up to the residences, 52 parking spaces and other ancillary spaces. 43 parking spaces would be for residents, while 9 accessible parking spaces would be specifically catered for the Ambulatory Surgery Center located at the adjacent Building 2/3. This housing variant would provide 57 units, all of which would be designed as one-bedroom apartments. Please see Exhibit A for all figures and plans indicating Building 1 as a future residential building.

During the interim period prior to construction of Building 1, the site would be utilized as a construction staging and coordination area, storing temporary construction trailers and construction materials and equipment. During this period, the 9 accessible parking spaces to serve the Ambulatory Surgery Center located at the adjacent Building 2/3 would be installed.

3. Building 2/3.

Sitting on a 2.9-acre fully landscaped parcel, the existing 7-story (plus penthouse) empty shell building is approximately 270,700 gross square feet and was initially designed to accommodate future research and development laboratories and associated support space. The Hollis Street Medical Office Building (MOB) project scope would include interior build-out to convert the existing building to a MOB and Ambulatory Surgical Center. In addition to a fully finished/mostly glazed façade, the existing building is equipped with a finished entrance lobby/café area. The building is also served with a 3-bay loading dock and Back of House (BOH) support, 4 public elevators, 2 service elevators, 2 exit stairs, mechanical shafts, Electrical/TR rooms, and a mechanical penthouse. The main entrance lobby to the building is being relocated from Hollis Street (East) to Chiron Way (West). Entry would occur through a pedestrian plaza which would include a vehicular drop off from Chiron Way.

The MOB would include Urgent care, imaging, blood draw/specimen collection lab and a retail pharmacy on the entrance level. Urgent care would have separate/dedicated access from the exterior. There would be an Ambulatory Surgery center, Sterile processing services, a patient-oriented conference center, a non-oncology infusion center and associated pharmacy. The MOB would also house an array of specialty clinics including women's clinic, OB/GYN, Prenatal, Pediatrics, Urology, Neuroscience, ENT & Surgery clinics.

The MOB is anticipated to have 107 physicians, 489 full time daily employees and a daily patient/visitor population of 4,655 people. The MOB would operate from 7:00 AM to 7:00 PM Mondays through Fridays, from 8:00 AM to 12:00 PM on Saturdays, and would remain closed on Sundays.

4. Building 4.

Building 4 is an existing 7-story, approximately 292,500 square feet building originally designed to accommodate research and development laboratories. The EMC Project would include interior tenant improvements to convert Building 4 to accommodate ambulatory care and associated support space on all levels.

Building 4 is anticipated to have 134 physicians, 509 full time daily employees and a daily patient/visitor population of 5,829 people. The building would operate from 7:00 AM to 7:00 PM Mondays through Fridays, from 8:00 AM to 12:00 PM on Saturdays, and would remain closed on Sundays.

5. Building 8.

Building 8, the new hospital facility, is planned as a 15-story building with an additional mechanical level and helipad, totaling approximately 785,000 square feet and a height of 330 feet. The hospital would include up to 325 beds, labor and delivery, a neonatal intensive care unit, postpartum, medical-surgical, an intensive care unit, emergency room, and operating rooms.

The Emeryville hospital would operate 24 hours every day in 3 shifts – 7:00 AM to 3:30 PM, 3:00 PM to 11:30 PM and 11:00 PM to 07:30 AM. Based on the existing operation of the Alta Bates Hospital in Berkeley, it is anticipated that approximately 45% of employees and physicians will work from 7:00 AM to 3:30 PM, 35% from 3:00 PM to 11:30 PM and 20% from 11:00 PM to 07:30 AM.

There will be an estimated total of 2,049 personnel, which includes 163 physicians and 1,886 full time employees to support continuous 24/7 operations. The total number of daily personnel on site is expected to be approximately 1,464 ($2,049/1.4$), including 116 physicians and 1,348 employees, reflecting a 5-day work schedule ($7 \text{ days} \div 5 \text{ days}$). This would result in approximately 52 physicians and 606 employees during the peak shift from 7:00 AM to 3:30 PM, 41 physicians and 472 employees from 3:00 PM to 11:30 PM and 23 physicians and 270 employees from 11:00 PM to 7:30 AM.

Building 8 is anticipated to have patient/visitor population of 488.

6. Helipad.

The EMC Project would include a helipad located on the roof level of the hospital, designed to support medical operations. It would accommodate one helicopter up to the size of a Sikorsky UH-60 Black Hawk. Currently Air Medical Operators utilize a variety of single and light twin helicopters in Northern California. These include the Bell 407, Eurocopter H125, H130, H135, and H145 platforms. Sutter anticipates that all or nearly all flights will be performed by these aircraft. Public Operators regionally utilize other, sometimes larger helicopters (Medium Duty Twin) that are not frequently employed for medical transportation but are included in major disaster plans. These include the Eurocopter MH-65 (US Coast Guard) and several variants of Sikorsky UH-60 (US Military, CalFire, US Coast Guard). While the helipad would be engineered to support these larger helicopter models to ensure flexibility, the intended day-to-day use is for smaller medical helicopters typically deployed for hospital transport.

It is anticipated to accommodate approximately one flight per day, primarily serving patient transport. The estimate of one helipad trip per day was developed using a conservative analysis of expected patient transfers based on regional sending hospitals, distance, traffic conditions, and clinical need. The team reviewed historical transfer patterns and applied professional judgment from experience with similar hospitals.

For planning purposes, we developed assumptions of helicopter usage based on historical patterns, geographic distance, clinical acuity and time sensitivity and traffic conditions. More distant hospitals were assumed to rely on air transport more frequently, hospitals at a moderate distance to use a helicopter roughly half of the time, while close hospitals were assumed to use ground transport.

These assumptions were intentionally conservative and benchmarked against other Sutter Health hospitals. It is noted that there is significant variation in hospital helipad usage due to hospital capability & mission profile (trauma centers have a significantly higher usage) and geography (higher usage at tertiary/quaternary hospitals that serve more rural areas). Both data sets suggest that usage could be lower, but planning for one trip per day allows for growth in volume over time. More information from other Sutter hospitals is provided on the next page.

Hospital	Total Bed Size	Landings (Annual)	Comments
Eden	130	52	This is a level II trauma center and interventional stroke center, and in similar geographic area as Alta Bates Medical Center and Emeryville.
Mills Peninsula Medical Center	301	110	This is a tertiary regional hospital with a high-level interventional stroke program and interventional cardiac and cardiac surgery program.
Solano	106	30	This is a community hospital with mostly outbound transportation needs for patients.
Santa Rosa	124	298	This is a regional tertiary hospital with interventional cardiac and NICU services, but is in a very different geographic area with much more extensive helicopter transport from rural Sonoma, Mendocino, Lake and Humboldt counties. The hospital also receives a number of "scene" transports (directly from the 911 call scene) due to its cardiac services and geographic location). Even though this is still below the conservative 1 transport per day estimate, we would not anticipate this volume in Emeryville.
Sacramento	523	550	This is a large, quaternary adult and children's hospital with extensive specialty programs for adult, pediatric and neonatal patients populations in Northern California, particularly for the rural communities to the North and East. We do not anticipate that volumes would be similar for Emeryville.
Roseville	418	354	This is a busy level II trauma center and otherwise tertiary regional hospital with several specialty programs. 93% of the helicopter landings at this facility are attached to "scene" calls where a helicopter transports directly from a 911 call and, for this facility, and largely associated with trauma and cardiac diagnosis from the largely rural areas to the north and east of this hospital. We would not anticipate the same factors in Emeryville.
Amador	52	32	This is a small, rural community hospital with almost exclusively outbound needs.

7. PDU Building.

The PDU Building is a vacant existing 20,700 square feet building and is proposed to be demolished. See Section 10 – Site Circulation (Shuttles Circulation and Off-Site Parking Facilities) for proposed use.

During the interim construction period after demolition, but prior to installation of the proposed new shuttle pick-up and drop-off location described in Section 10, the PDU Building site would be utilized as a construction staging and coordination area, storing temporary construction trailers and construction materials and equipment.

8. Building 13/14 (Parking Garage).

Building 13/14 is a dedicated parking structure accommodating vehicle and bicycle parking, a café of approximately 1,200 square feet and a bicycle repair kiosk. Access to the building is provided from two driveways on Horton Street. Spaces that allow electric vehicle charging are provided in amounts required by California Building Code (59 EV stalls), and the building is solar-ready. The building height is stepped back from the adjacent Sherwin-Williams Mixed Use Project. The east side that faces Sherwin-Williams is four stories plus roof-top parking (5 levels of parking) and is set back 37 feet from the adjacent Sherwin-Williams building. The west side that faces Horton Street is 10 stories plus roof-top parking (11 levels of parking). The 10-story portion of the garage is set back 81 feet from the closest Sherwin-Williams building.

The north end of the garage accommodates an amenity space that is planned to house a café or similar food service business. A bicycle repair kiosk would be located in the parking structure.

The parking garage provides 1,991 spaces, which serves the entire EMC Project.

There are no proposed changes to Building 13/14.

9. Open Space and Recreation Amenities.

The PDP's revised conceptual plan for open space includes a network of grade-level, semi-public landscaped areas along building frontages, complemented by privately owned, publicly accessible open spaces such as Stanford Plaza, Chiron Plaza, Horton Plaza, Hollis Green Park and a pedestrian Paseo. With the addition of two new buildings (Building 1 and Building 8) and the demolition of the PDU building, the open space design would be refined to enhance user experience. The proposed design would introduce a publicly accessible, at-grade landscaped circulation system connecting the buildings, with a strong emphasis on pedestrian-oriented design. This system would integrate planting areas, water-treatment features, drop-off zones, and building access points.

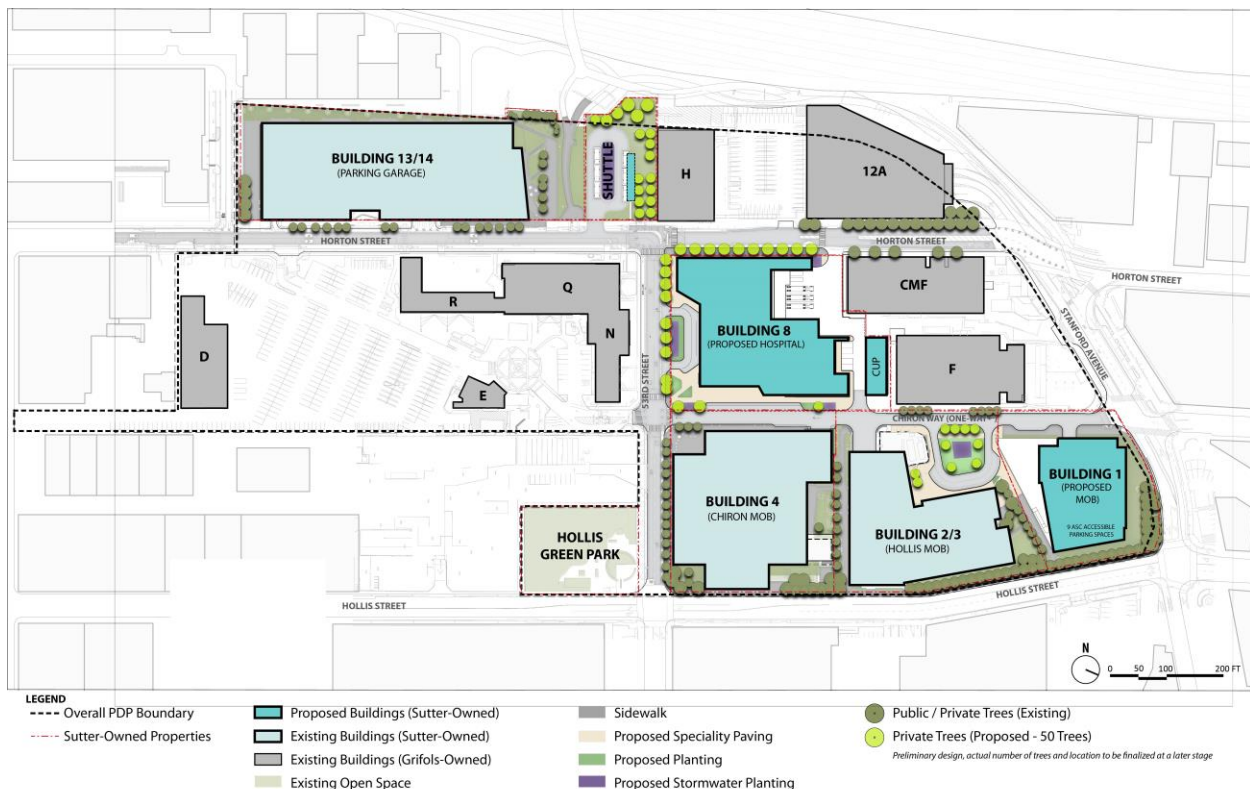


FIGURE 5: CONCEPTUAL OPEN SPACE / LANDSCAPE PLAN

As shown in Figure 5 above, the new Stanford Plaza would occupy the northeastern corner of the PDP site, on a portion of the proposed Building 1 lot at the intersection of Stanford Avenue and Hollis Street. The proposed Building 1 and existing Building 2/3 (5555 Hollis) would be separated by Chiron Plaza, which would also include a drop-off area serving the converted MOB at 5555 Hollis Street. The paseo along the southern edge of 5555 Hollis would continue to provide a pedestrian friendly corridor from the existing Building 4 (5300 Chiron Way).

Building 13/14 is a parking garage that includes Horton Plaza, an at-grade level patio area. Horton Plaza is immediately adjacent to Horton Landing Park and directly connected to the South Bayfront Bicycle & Pedestrian Bridge. These plazas provide outdoor seating, pedestrian pathways, and gathering spaces, and remain available for public use.

The hospital (Building 8) would incorporate landscaping elements, bioretention planters and establish a pedestrian connection linking the parking garage (Building 13/14) to 5300 Chiron Way (Building 4), 5555 Hollis (Building 2/3), and the future Building 1.

Development of the hospital would require removal of approximately 16 existing public trees within the Building 8 site, due to site constraints, including limited available area, necessary driveway access and a building footprint that is maximized to accommodate the required program on Level 1.

To offset this impact, new trees would be planted at the proposed shuttle stop area adjacent to Horton Plaza. The additional trees would provide shade for pedestrians and enhance the experience of the visual corridor coming off the South Bayfront Pedestrian Bicycle Bridge.

Construction of the new plazas would be phased to align with the development of the building adjacent to each open space. Sutter would also continue to operate and maintain the current 30,000-square-foot Hollis Green Park in accordance with the existing conditions of approval and the Grant of Easement and Agreement recorded July 6, 2007. The park's entry gates would remain open to the public from 8:00 a.m. until sunset.

The ultimate goal is for these privately owned, publicly accessible open courtyards and pathways to function as part of a broader open space network that would guide pedestrians and bicyclists from residential areas north and east of the PDP site to the South Bayfront Bicycle & Pedestrian Bridge, Bay Street and the waterfront pathways. This network would help advance the vision outlined in the City of Emeryville General Plan, particularly Chapter 4 – Parks, Open Space and Public Services. Collectively, the open spaces distributed throughout the campus would enhance the connectivity, livability and overall comfort for the campus users and the broader community.

10. Site Circulation.

i. Removal of Traffic Diverter on 53rd Street / Horton Street

The proposed project is anticipated to generate over 20,000 daily trips, including approximately 1,500 trips during the peak hour. A traffic diverter currently located on the north side of the 53rd Street/Horton Street intersection restricts through movements along Horton Street and prohibits westbound right turns from 53rd Street to northbound Horton Street. As a result, all vehicular access to and from the Horton Street garage is funneled via 53rd Street, that would constrain circulation for the proposed hospital and medical office buildings (MOBs) and contribute to congestion on Hollis Street, Powell Street, and 40th Street.

Initial Purpose of the Existing Traffic Diverter for Prior Development Project

The traffic diverter was installed pursuant to a development agreement with BioMed Realty for the Emery Yards project. It functions to limit vehicle traffic on the Horton Street Bicycle Boulevard. Horton Street is designated as a Bicycle Boulevard between 53rd Street and 40th Street in the City's Active Transportation Plan.

To address increased traffic volumes along Horton Street, particularly between the new office buildings and the Horton Street parking garage, separated bikeways were constructed on both sides of 53rd Street between Horton Street and Hollis Street, as well as on both sides of Horton Street between 53rd Street and 45th Street. These improvements were implemented by BioMed Realty to reduce effects on bicycle travel along the corridor.

South of 45th Street, Horton Street continues to operate as a bicycle boulevard. Vehicles exiting the Horton Street parking garage are currently restricted to left turns only, helping maintain low traffic volumes and preserve bicycle boulevard operations south of 45th Street.

How the Existing Traffic Diverter affects Proposed Development

Under existing plus project conditions, with the traffic diverter, preliminary traffic simulation analysis shows significant queuing during the AM peak hour on Horton Street, 53rd Street, Hollis Street, Powell Street, and 40th Street for vehicles accessing the garage (see Figure 6). During the PM peak hour, all exiting vehicles would be required to turn left onto Horton Street (due to the current driveway configuration and right-turn restrictions), followed by a right turn onto 53rd Street, and then a left or right turn onto Hollis Street to exit the campus. Additionally, the proposed reduction of westbound 40th Street from two lanes to one as part of the 40th Street multimodal improvements, which would require increased green time for westbound traffic, is expected to result in extended queues on southbound Hollis Street. These queues could extend back to Powell Street and spill onto adjacent side streets, potentially creating gridlock conditions (see Figure 7).

Proposal to Remove Traffic Diverter – Improved Circulation and Potential Impacts

To improve site access and circulation, the project proposes removing the existing traffic diverter at the 53rd Street/Horton Street intersection. Traffic signal modifications would be implemented to accommodate through vehicle movements, as well as pedestrian and bicycle crossings. Additional modifications at the Horton Street garage would allow vehicles to turn right onto southbound Horton Street, improving access to 40th Street. The analysis indicates that removing the diverter and allowing right turns from the garage would significantly enhance traffic

operations, resulting in shorter queues and improved overall circulation, as shown in Figures 8 and 9. However, allowing vehicles exiting the parking garage to turn right onto southbound Horton Street would also increase traffic volumes along Horton Street and adversely affect its function as a bicycle boulevard.

The traffic signal modifications and multimodal improvements associated with the removal of the traffic diverter at the 53rd Street/Horton Street intersection, and the improvements needed to reduce effects on bicycle travel along the Horton Street Corridor, would be coordinated with the City through the project development review and approval process.

Measures to Reduce Potential Impacts

To reduce potential conflicts among bicyclists, pedestrians, and vehicles near the hospital driveways on 53rd Street and the shuttle access driveway on Horton Street, the project will incorporate design features that prioritize pedestrian and bicycle movements, along with signage and pavement markings to alert all users to potential conflict areas.

Shuttle Access Driveway/Horton Street - To reduce effects on traffic operations at the 53rd/Horton Street intersection, the shuttle access driveway would be restricted to right-turns only.

Stanford/Horton Intersection - The project will implement pedestrian and bicycle crossing improvements focusing on increasing visibility, reducing vehicle speeds, and reducing conflict points, such as through protected intersection design elements (corner islands), colored pavement markings (green paint), setback crossings, and raised crossings in coordination with the city.

Stanford/Chiron Way/Peladeau Street - The project will implement an all-way stop control at this intersection to address inadequate sight distance issues for traffic on Peladeau Street. In addition, the project will implement pedestrian and bicycle crossing improvements focusing on increasing visibility, reducing vehicle speeds, and reducing conflict points, such as through protected intersection design elements (corner islands), colored pavement markings (green paint), setback crossings, and raised crossings in coordination with the city.

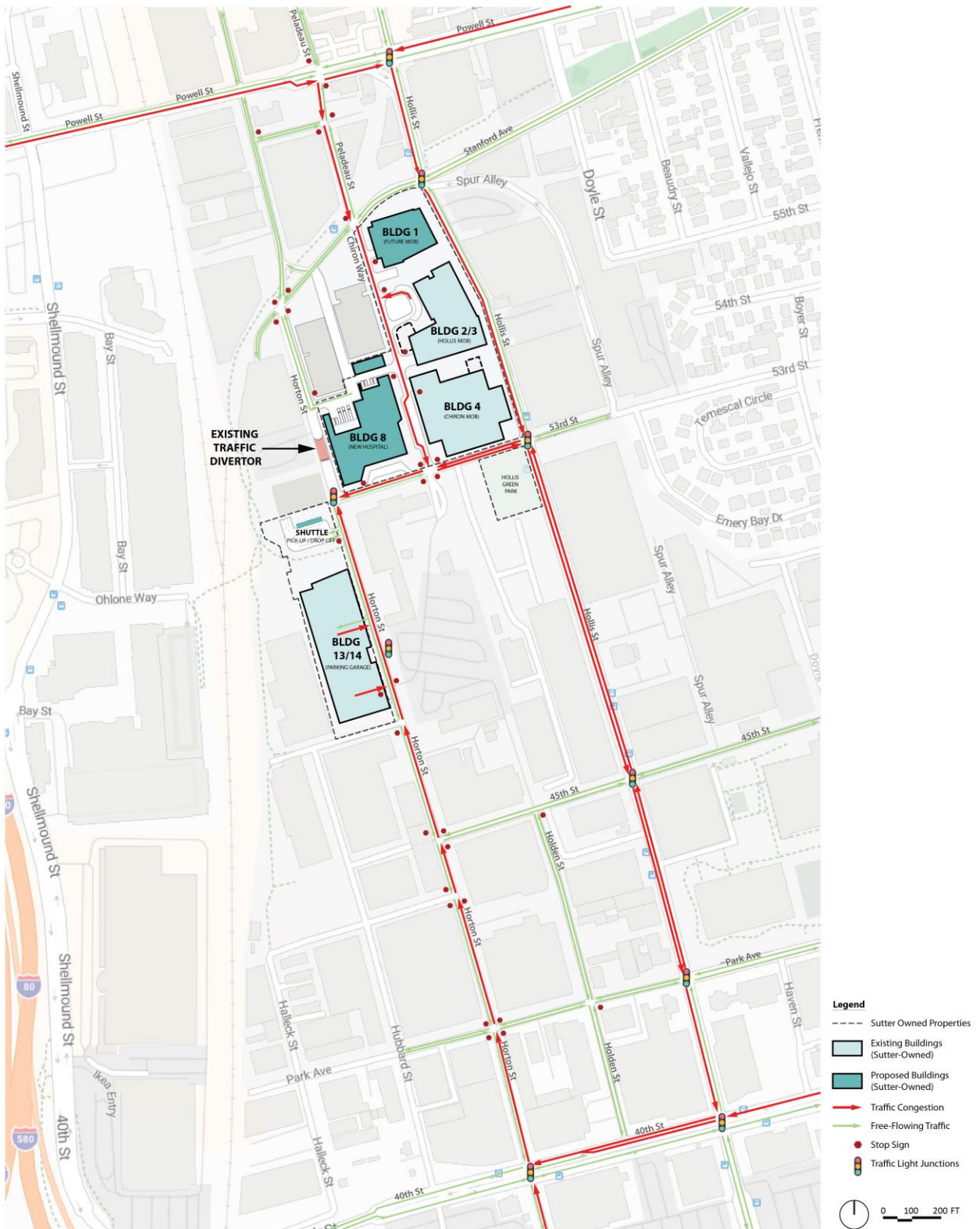


FIGURE 6: VEHICULAR TRAFFIC CONGESTION – EXISTING PLUS PROJECT AM
(WITH EXISTING TRAFFIC DIVERTER)

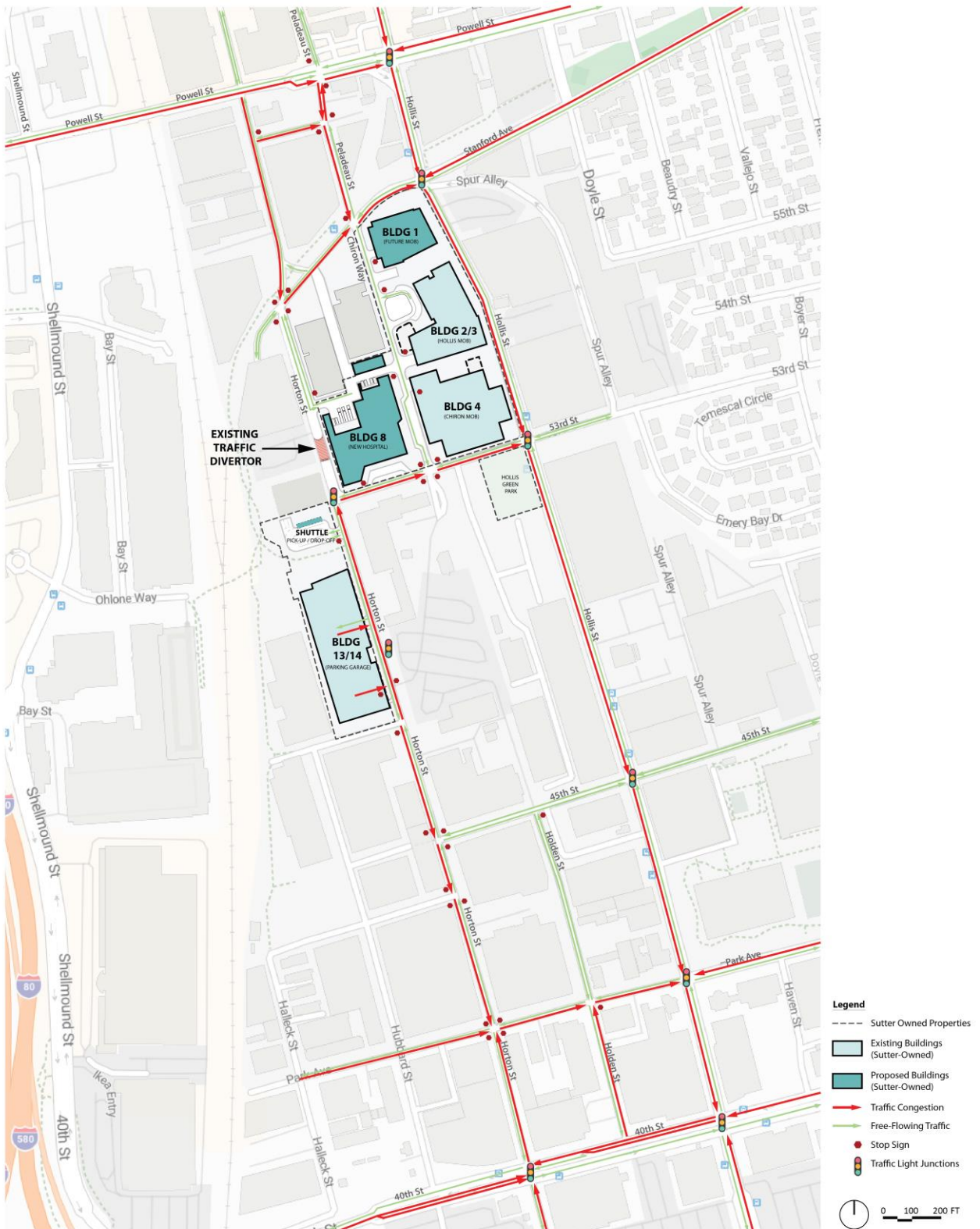


FIGURE 7: VEHICULAR TRAFFIC CONGESTION – EXISTING PLUS PROJECT PM
(WITH EXISTING TRAFFIC DIVERTER)

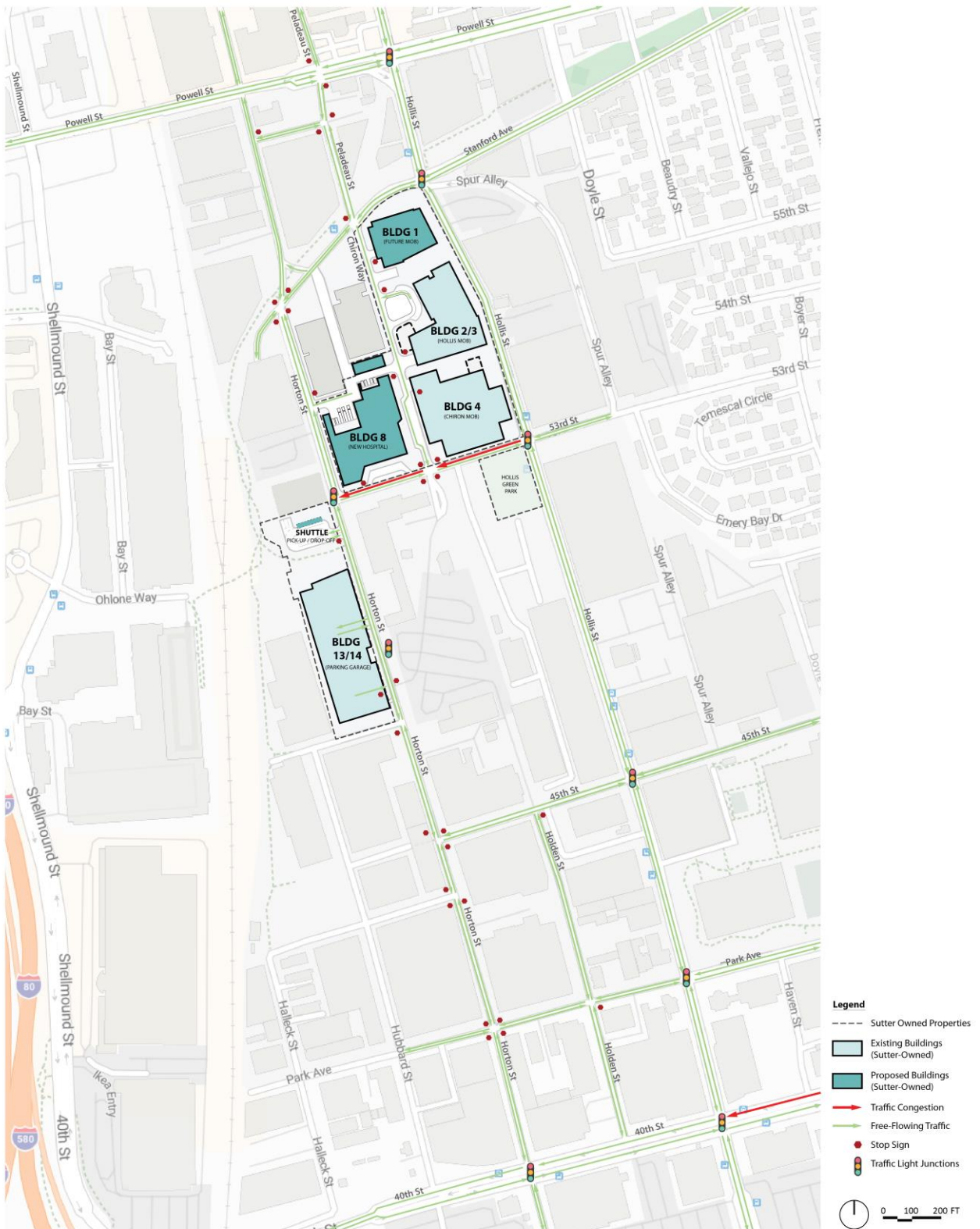


FIGURE 8: VEHICULAR TRAFFIC CONGESTION – EXISTING PLUS PROJECT AM
(TRAFFIC DIVERTER REMOVED)

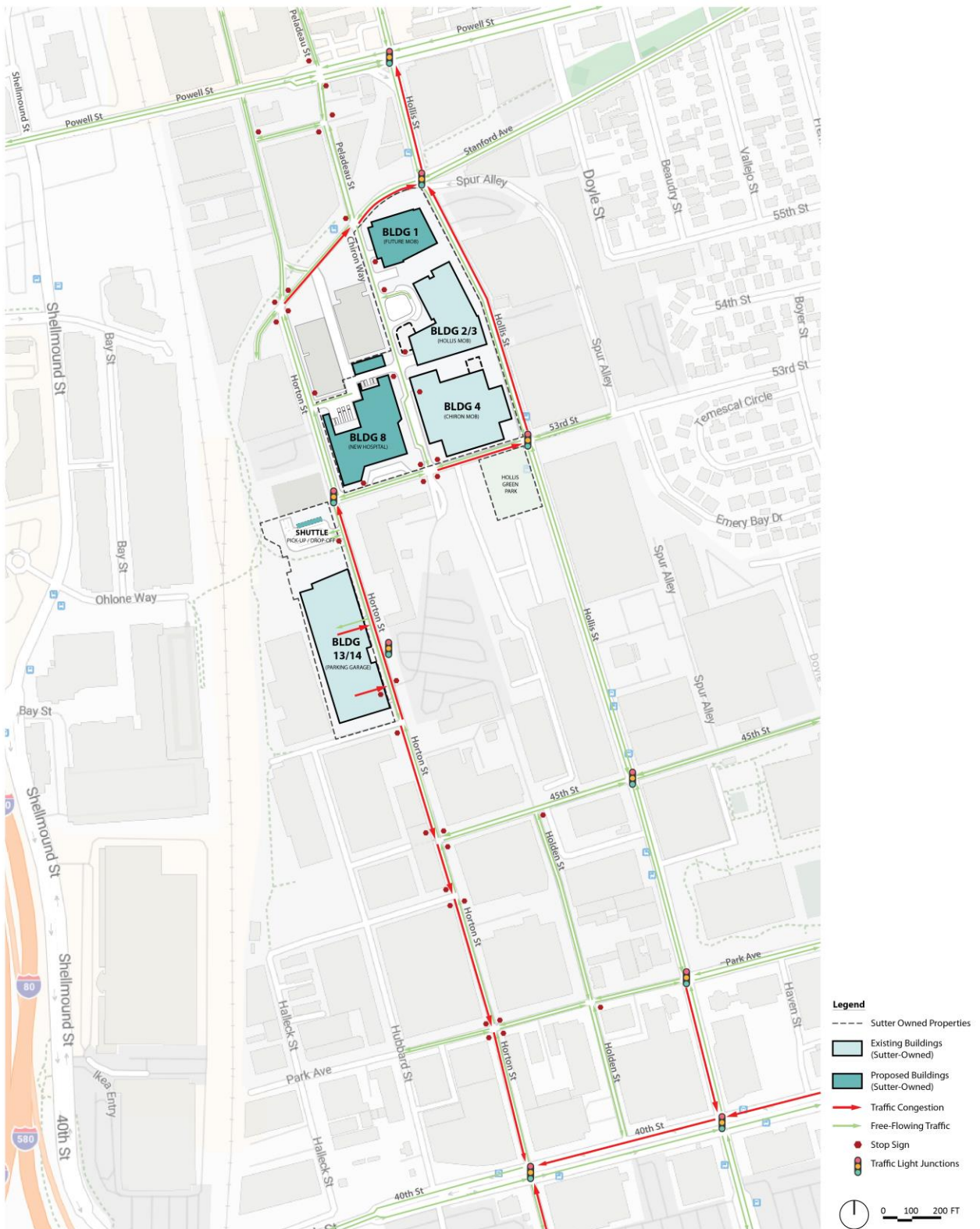


FIGURE 9: VEHICULAR TRAFFIC CONGESTION – EXISTING PLUS PROJECT PM
(TRAFFIC DIVERTER REMOVED)

ii. Bicycle Circulation

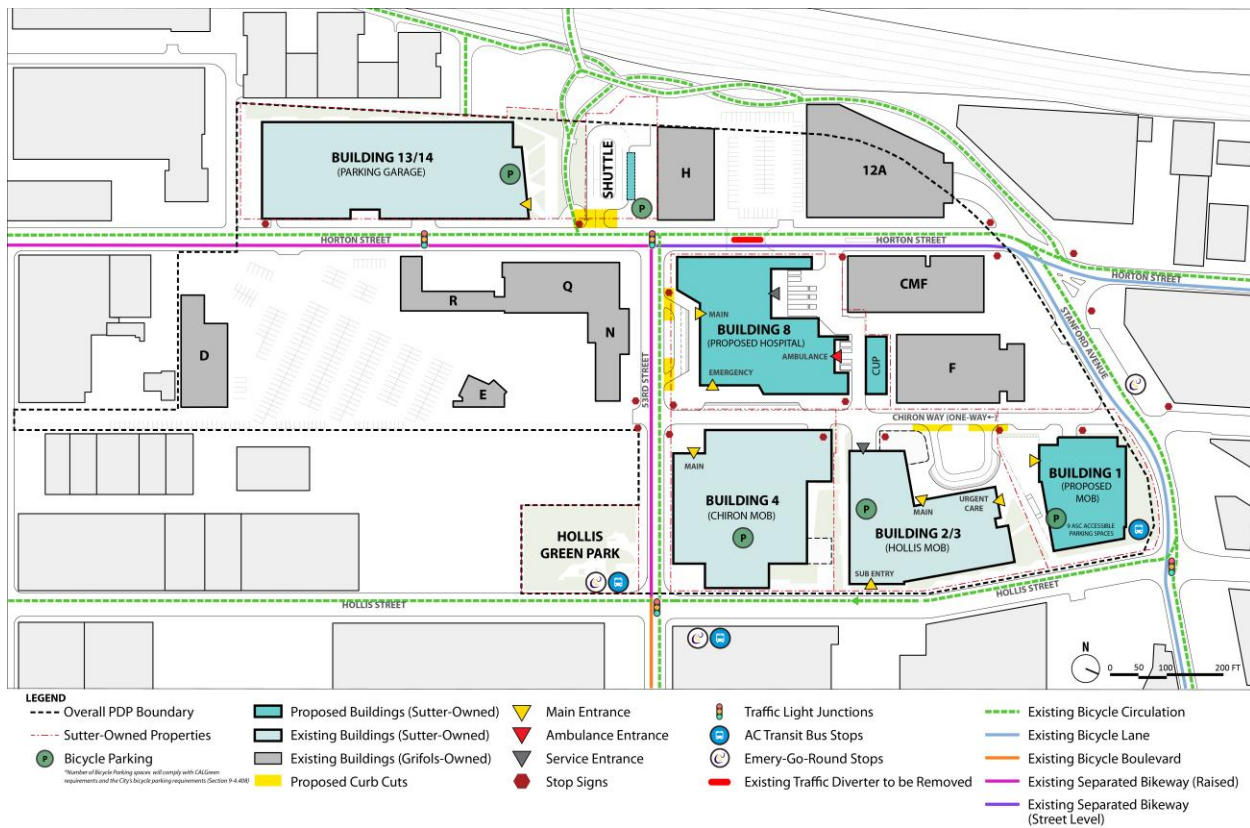


FIGURE 10: PROPOSED BICYCLE CIRCULATION

Bicycle facilities in the project vicinity include shared-use paths, bicycle lanes, buffered bicycle lanes, bicycle routes, bicycle boulevards, and separated bikeways. The City of Emeryville recently completed separated bikeways (located at sidewalk level) on Horton Street between 45th and 53rd Streets and on 53rd Street between Horton and Hollis Streets. The Horton Street bikeway provides a connection for cyclists traveling to the Emeryville Amtrak Station.

Bicyclists traveling along Horton Street between 45th Street and Stanford Avenue would continue to be separated from vehicular traffic by the raised cycle track (at sidewalk level) between 45th Street and 53rd Street and the protected cycle track between 53rd Street and Stanford Avenue. The protected cycle track between 53rd Street and Stanford Avenue remain at street level and is separated from vehicular traffic by bollards.

In addition, the project would provide bicycle parking within the site. Long-term and short-term bicycle parking are provided at the existing parking garage (Building 13/14) and 5555 Hollis (Building 2/3) and would also be included at the proposed shuttle stop as well as at Building 1, with sufficient bicycle parking spaces to meet CALGreen requirements and will also comply with the City's bicycle parking requirements (Section 9-4.408).

Bicycle Network Connections and Alternative Routes

The EMC Project Site also connects to Horton Landing Greenway, a multi-use path developed by the City as part of its South Bayfront Bicycle & Pedestrian Bridge and Horton Landing Park project. These paths provide connections to the pedestrian bridge and Bay Street to the west.

They will also offer access to the EMC Project site and other Emeryville neighborhoods to the east. The paths link Emeryville Greenway and the Amtrak station to the north, and the Bay Trail to the south.

The path runs in a north-south direction, parallel to and west of Horton Street along the railway tracks, connecting Sherwin Avenue and Stanford Avenue and providing a route without conflicts with vehicular traffic. This alternative bicycle and pedestrian route will give users another way to travel without using Horton Street. Bicyclists may access these City paths from Horton Street and Horton Plaza.

Measures to Reduce Potential Impacts to Bicycle Travel

With the development of the project, curb cuts would be required on the west side of Horton Street, south of 53rd Street, to provide shuttle access. In addition, two curb cuts would be added on the north side of 53rd Street to accommodate inbound and outbound access to the hospital drop-off area. The project will coordinate with the City to evaluate potential improvements to the existing protected bike lanes on Horton Street between 53rd Street and Stanford Avenue to reduce effects on bicycle travel.

To also minimize potential conflicts between bicyclists and vehicles near the hospital driveways on 53rd Street and the shuttle access driveway on Horton Street, the project shall consider the following measures in coordination with the City:

- To emphasize pedestrian and bicycle priority, the sidewalk and separated bikeway shall be maintained as raised facilities across both driveway crossings, requiring vehicles to ramp up when crossing.
- The bikeways shall include high-visibility green pavement markings through the full width of each driveway crossing, to delineate the conflict zone.
- Driveway design shall minimize curb radii and crossing width to the maximum extent feasible, to reduce vehicle speeds.
- Sight distance at each driveway shall be maximized through the establishment of clear sight triangles, with restrictions on landscaping, and signage.
- Enhanced signage and pavement markings shall be installed, including “Yield to Bicycles” signage for exiting vehicles and advance warning signage for both motorists and bicyclists.

iii. Pedestrian Circulation

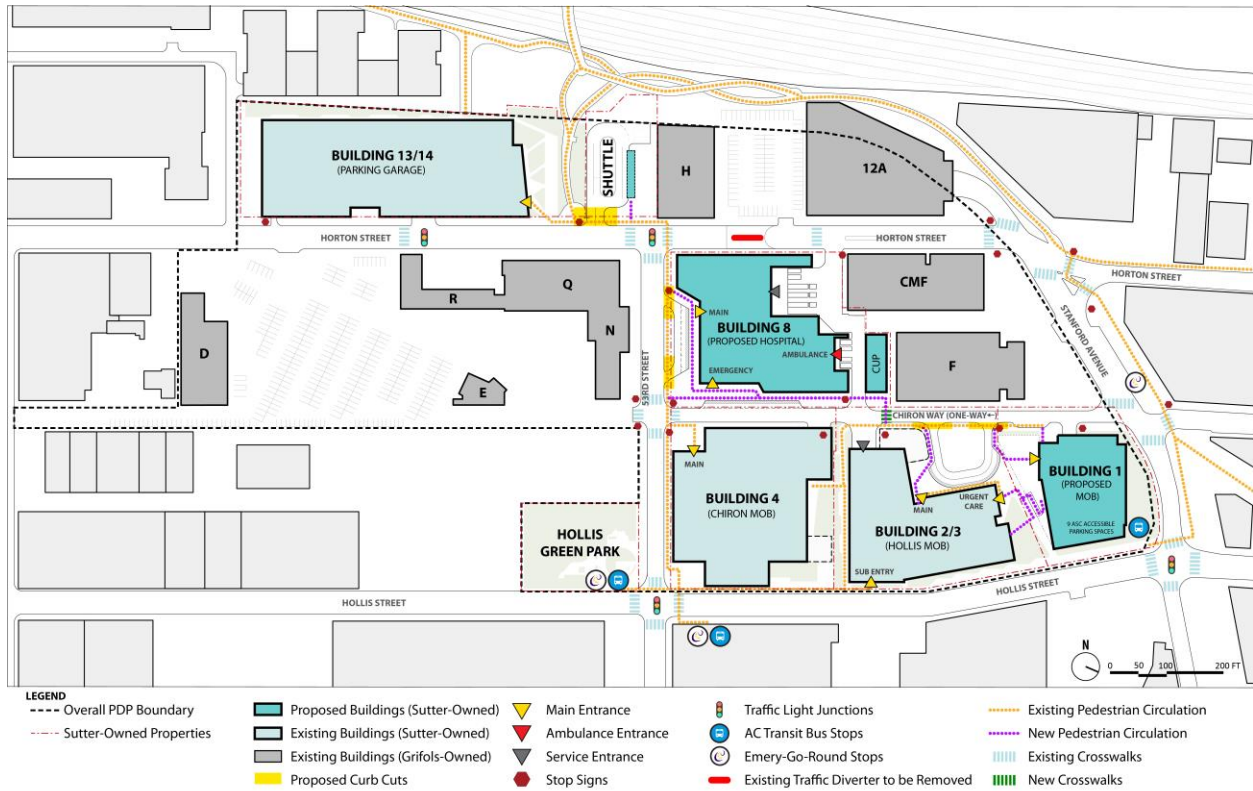


FIGURE 11: PROPOSED PEDESTRIAN CIRCULATION

Sidewalks and crosswalks are provided along all roadway segments in the project area except on the west side of Chiron Way, from south of the Grifols building to 53rd Street. The project would construct a new sidewalk along the hospital frontage on the west side of Chiron Way, extending from 53rd Street to the current sidewalk terminus just north of the proposed hospital site. The project would also install a midblock crossing on Chiron Way to facilitate pedestrian crossings between the hospital and the MOBs. Pedestrian signal heads and push buttons are installed at nearby signalized intersections, while high-visibility or standard crosswalks are present at nearby unsignalized intersections. Continuous pedestrian facilities would connect the site with surrounding land uses, including nearby restaurants and transit stops. The project would also provide continuous pedestrian connections between the on-site shuttle plaza, the Horton Street parking garage, the hospital, and the medical office buildings.

iv. Shuttles Circulation and Off-Site Parking Facilities

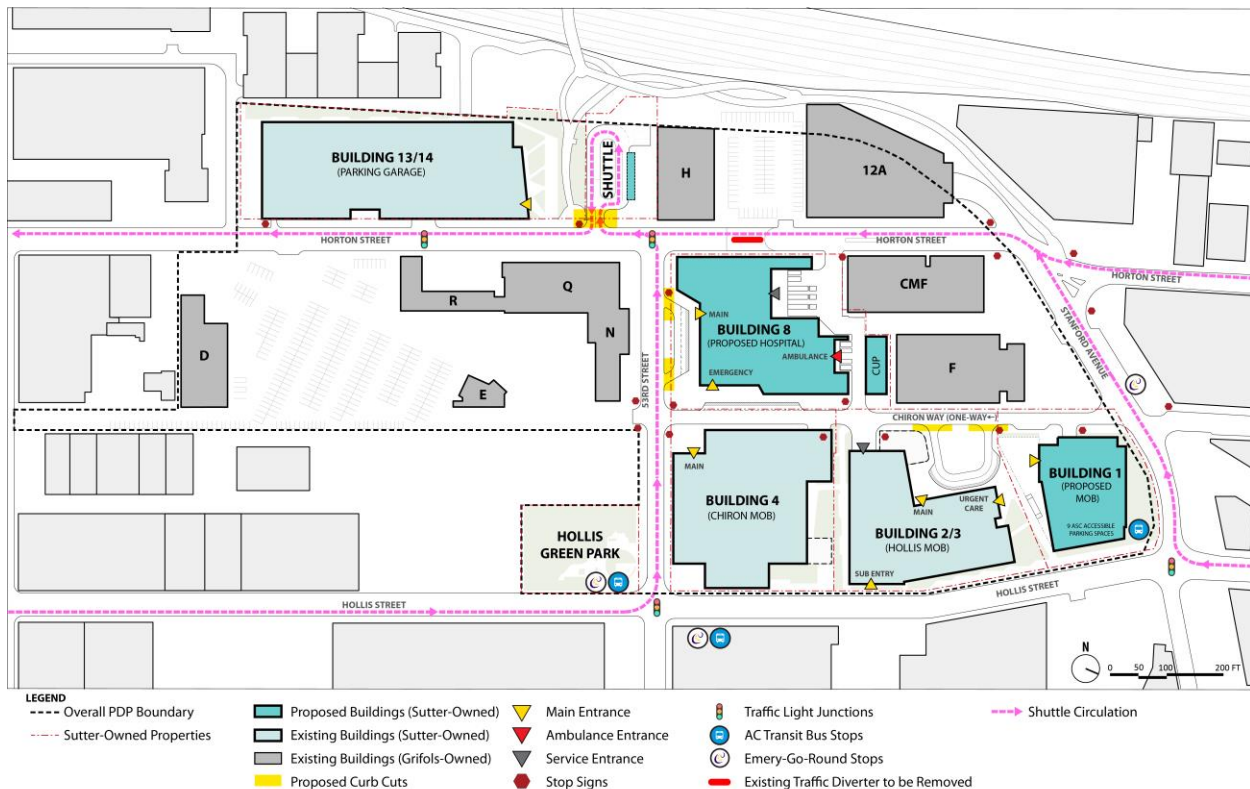


FIGURE 12: PROPOSED SHUTTLE CIRCULATION

To promote transit use, the project would operate a dedicated shuttle service to transport staff and patients/visitors between the Emeryville Medical Campus and the Emeryville Amtrak Station, MacArthur BART Station, West Oakland BART Station and off-site parking facilities. The shuttle service would be available for use by the public and would begin running once the Tenant Improvement work for the Medical Offices are completed. Shuttle loading and unloading will only occur in the new Shuttle Stop at the PDU Site.

The removal of the PDU building allows for a new shuttle pick-up and drop-off location. The project would need to operate approximately 11 shuttles (with 20 passenger capacity) during employee peak hours to transport employees between Berkeley satellite parking lot, the Oakland satellite parking lot and the campus. During employee peak hours between 6:00 AM to 8:00 AM and between 3:00 PM to 6:00 PM, there would be a frequency of 1 shuttle every 5 minutes. During off-peak hours, the frequency of shuttles would potentially be 1 shuttle every 20 minutes.

The project would require additional shuttles with a larger passenger capacity to transport employees and visitors to and from the Emeryville Amtrak Station, MacArthur BART Station, West Oakland BART Station and the campus. Potentially, there will be a frequency of 1 shuttle every 15 minutes during both peak and off-peak hours, to the Amtrak Station and the 2 BART stations.

The anticipated shuttle frequencies may be adjusted as needed to accommodate actual demand. The shuttles to the off-site parking facilities would operate after the full buildout of the campus. However, the BART and Amtrak shuttles may need to operate before that to encourage 5555 Hollis MOB employees to use alternative transportation modes even if there is no parking deficit.

The demolition of the PDU building and construction of the new shuttle stop provides an opportunity to enhance the arrival experience for pedestrians coming off the pedestrian bridge. A perimeter fence required for security purposes could be designed as a decorative greenwall or landscaped fence, providing a soft separation between the park and the shuttle stop.

The project would also provide off-site parking facilities for employees and operate a dedicated shuttle service to transport staff between the off-site parking locations and the EMC campus. These off-site parking facilities would potentially be located at Berkeley Alta Bates to the north of the project site and the Oakland Summit Campus to the south of the project site. These shuttle services would be mainly used for bringing employees from the off-site parking facility to the campus. This satellite parking would not be needed until the hospital is operational. Refer to the parking analysis and trip generation memo for additional information.

v. Public / Visitor / Employee Vehicular Circulation

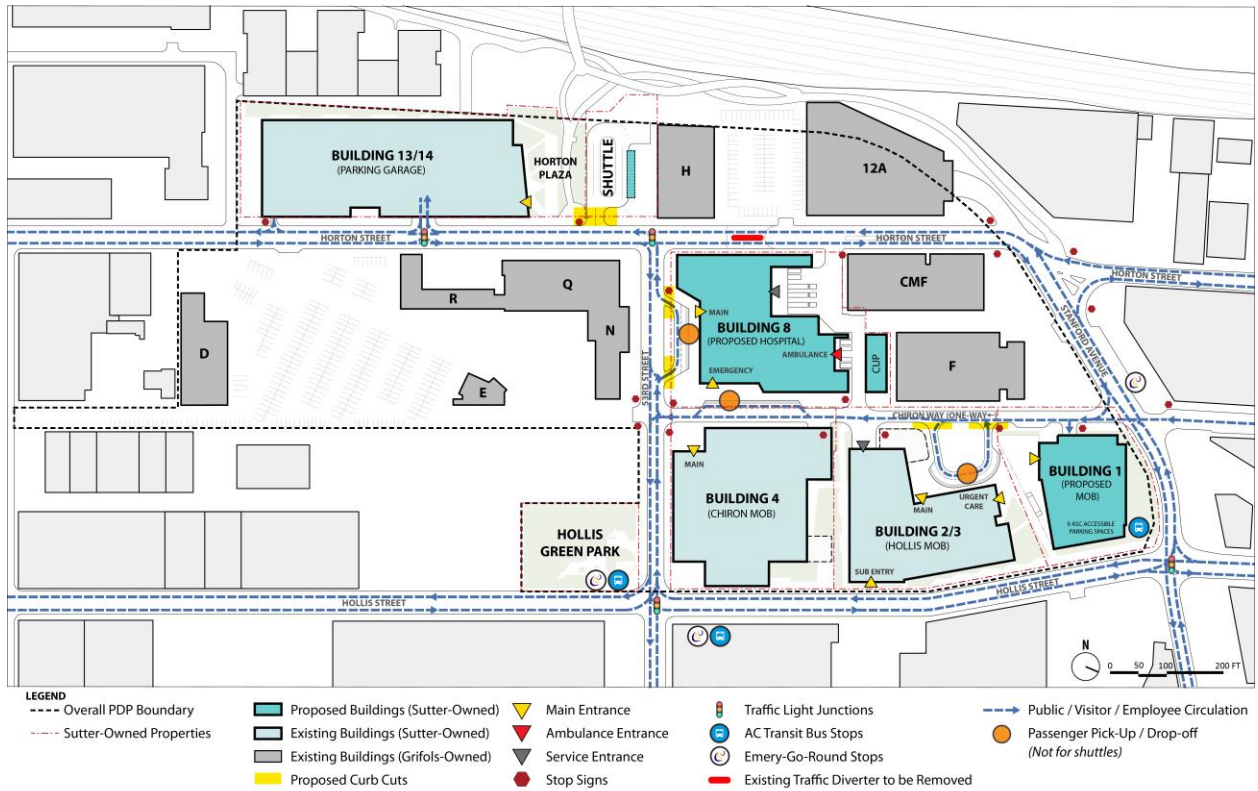


FIGURE 13: PROPOSED PUBLIC / VISITOR / EMPLOYEE CIRCULATION

The proposed EMC Project has public, visitor and employee vehicular circulation limited to the city street network along Hollis, Stanford, 53rd and Horton Streets, and circulation within the campus along Chiron Way. Designated passenger pick-up and drop-off locations mentioned in the TDM Plan are shown on Figure 13 above.

vi. Ambulance Circulation

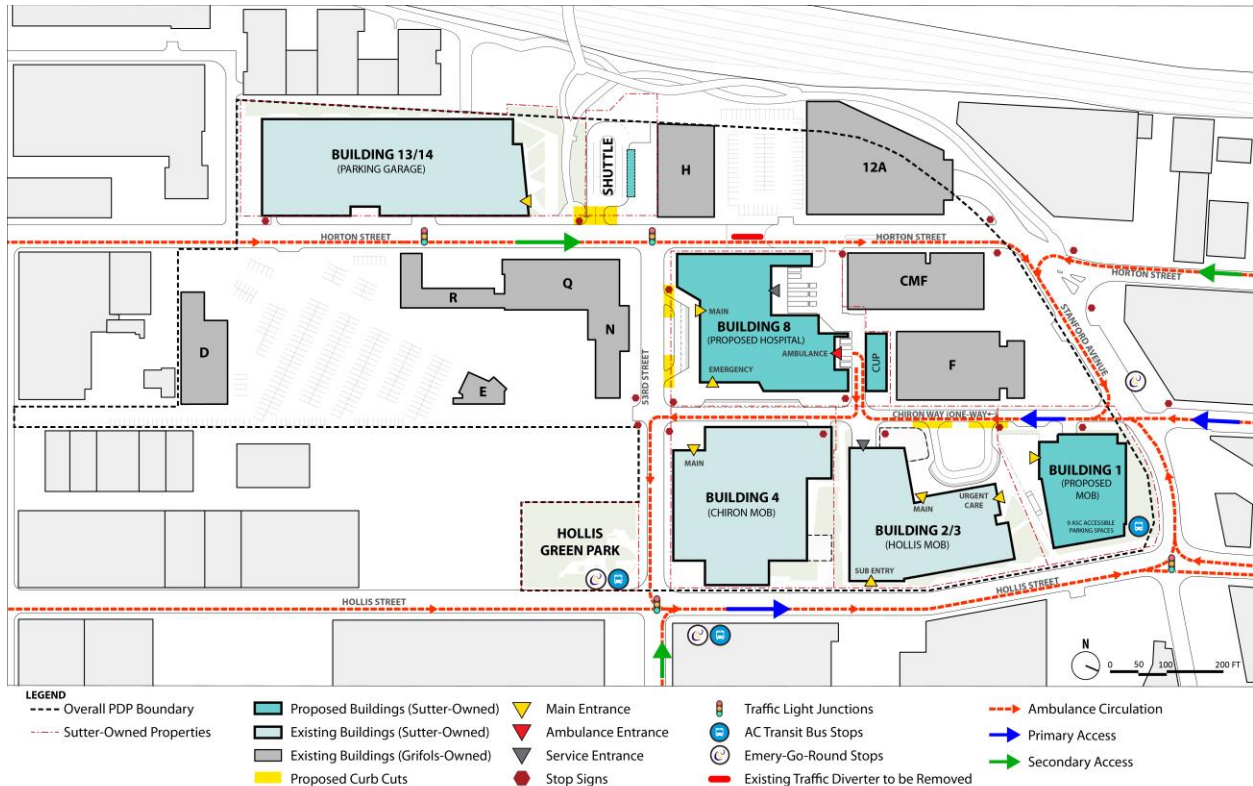


FIGURE 14: PROPOSED AMBULANCE CIRCULATION

The proposed Hospital is a specialized hospital providing labor and delivery services, neonatal care, and related inpatient and outpatient services. As such, emergency medical transport activity is expected to be less intensive than a full general acute-care trauma hospital.

Ambulances would access the hospital via Stanford Avenue and Hollis Street, then through Chiron Way as shown in Figure 14 above. Emergency access and routing would be coordinated with Alameda County EMS, local fire departments, and law enforcement to ensure safe and efficient response based on real-time traffic conditions and emergency protocols. Ambulance trips would occur on an as-needed basis and are expected to be intermittent throughout the day and night rather than on a fixed schedule.

Ambulances serving the hospital would not park on public streets. Ambulances are operated by third-party EMS providers and would use designated on-site emergency vehicle loading areas for short-term staging or patient transfers. The facility would not maintain a dedicated ambulance fleet, and the number of ambulances serving the hospital at any time would be determined by regional EMS dispatch protocols and patient needs. Based on current operations at Alta Bates Berkeley Hospital, the proposed Emeryville hospital is anticipated to receive approximately 20 to 30 ambulance arrivals per day. Ambulance operations would also comply with all applicable California Vehicle Code and local noise regulations. Sirens would be used only when necessary for public safety and would not be used continuously upon arrival or departure.

vii. Service Vehicles Circulation

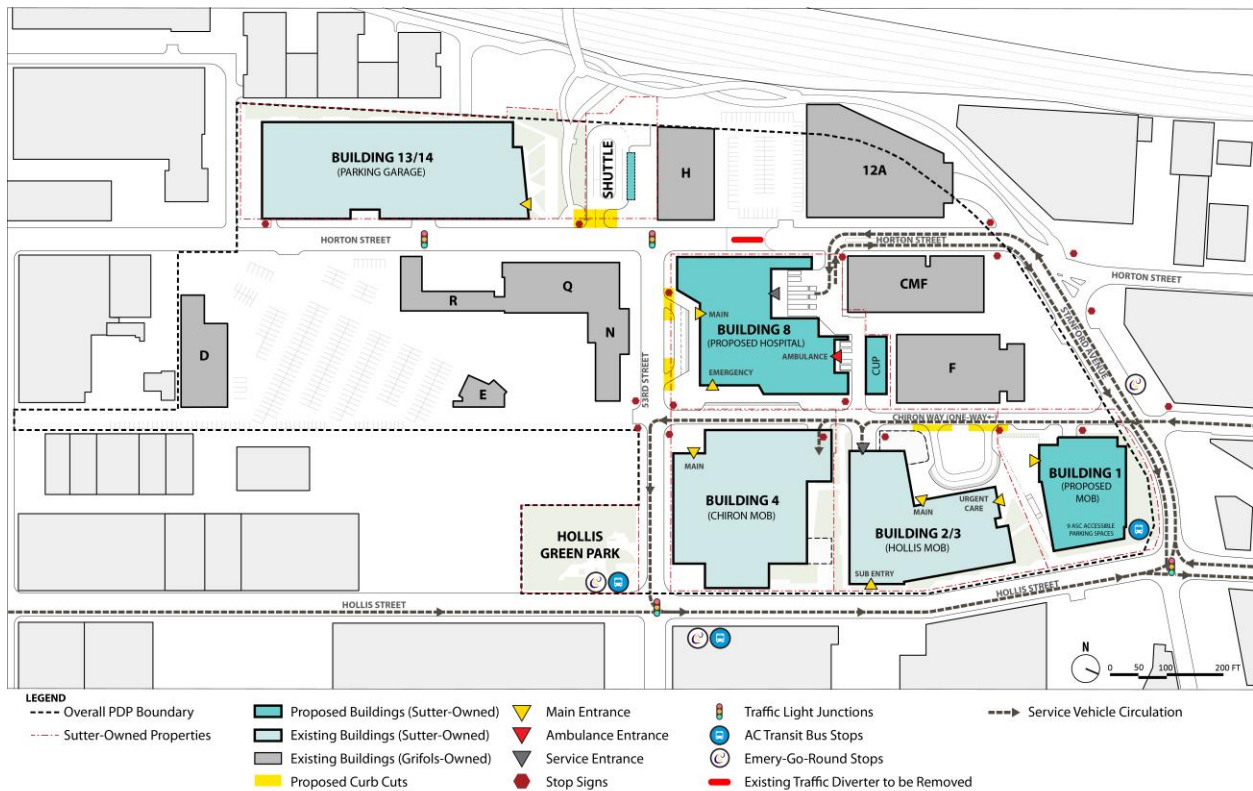


FIGURE 15: PROPOSED SERVICE VEHICLES CIRCULATION

Service vehicles would only access the site through Stanford Ave to Chiron Way or Horton Street as shown in Figure 15 above. The plan is for these service vehicles to operate and complete major deliveries before the early-morning peak hour traffic, and limited deliveries would occur during the day.

The truck delivery data was analyzed for the Alta Bates Ashby Hospital to as reference for estimated frequency and times of deliveries for the new Emeryville hospital.

VENDOR	FREQUENCY	DAY/TIMES	TRUCK SIZE
Supplements	Weekly	Varies	Box Truck
Vending	Bi-Monthly	Varies	Box Truck
Specialty Meats	Weekly	M	Box Truck
DISTRIBUTION ABSMC: Patient Meals	Daily	Herrick Meal Deliveries: Times: 0725, 0900, 1215, 1545	Box Truck
Seafood	Weekly	Varies	Box Truck
Linen	Weekly	Th	Box Truck
PEPSI	Weekly	F	Half-Semi
REYES COCA COLA	Bi-Weekly	W, F	Half-Semi
Uniforms	Weekly	Th	Box Truck
THE AMERICAN BOTTLING CO	Bi-Monthly	Th	Box Truck
Bakery	Weekly	M	Box Truck
Broadliner	Weekly	M, W, F	Half-Semi
Produce and Grab & Go	Weekly	M, Tu, W, Th, F	Box Truck
Specialty Snacks	Monthly	Varies	UPS
Office Supplies	Bi-Monthly	Varies	Box Truck
McKesson - Pharmacy	Daily	Times: 0730, 1400	Box Truck
Medline	Daily	0700, 1700	53' Semi, Box Truck
Fisher Scientific	3x a week	Varies	FedEx / UPS
O2	Bi-Monthly	Varies	53' Semi
Fuel Tank	Monthly	Varies	53' Semi
FedEx and UPS	Daily	Varies	FedEx / UPS

(Bi-Monthly = 2 times a month, Bi-Weekly = 2 times a week)

11. Utilities.

New utility services would be constructed to serve the proposed hospital building. Existing public infrastructure for water, storm drain, and sewer mains are available within the 53rd Street and Horton Street frontages adjacent to the hospital building parcel.

New domestic and fire water services are planned off of either the existing 12-inch main in Chiron Way or the existing 10-inch main in Horton Street, or a combination of both. All new water service applications would be processed through the utility owner, EBMUD, with oversight by the City.

Existing storm drain infrastructure surrounding the Hospital parcel includes an existing 13.5ft by 15ft underground concrete culvert within 53rd Street, an 18-inch storm main within Horton Street, and a 12" storm main within running east to west on the north side of the Hospital Parcel. The Culvert within 53rd is owned by the Alameda County Flood Control District while the 18-inch within Horton Street is under the jurisdiction of the City of Emeryville. The on-site storm drain is private and owned and maintained by Sutter Health. Two existing 18-inch storm drain laterals were constructed as part of the BioMed Realty public backbone infrastructure improvements to serve this development site. This eliminates the need to obtain new connection permits with the County by utilizing these existing services. The private 12-inch storm pipe located on-site within the northern portion of the hospital parcel is also available for on-site storm water discharge. Additional connections to this pipe can be permitted under site utility permit. A capacity analysis will be performed later to determine if new public storm drain infrastructure is anticipated to be needed in order to service the proposed development. Stormwater quality will be managed on-site with LID treatment facilities programmed and designed to meet State and Local water quality requirements. To facilitate access to the hospital from Chiron, several existing LID flow through planters currently treating the Chiron pavement will be reprogrammed within the hospital site, in addition to the new facilities required to manage the hospital parcels storm water runoff.

A new sewer service lateral for the hospital building is planned for the west side of the parcel and would connect to the existing 12-inch main within Horton Street. In addition, site sewer would connect to the existing private 10-inch pipe that flows west, parallel with the parcel's northern property line. This existing onsite sewer discharges to the existing 12-inch main within Horton Street. A capacity analysis will be performed later to determine if additional public regional sewer infrastructure is anticipated to be required in order to serve the project.

The project would not propose to extend the recycled water main location to the project site. Given the project's medical campus use, recycled water would be limited to landscape irrigation.

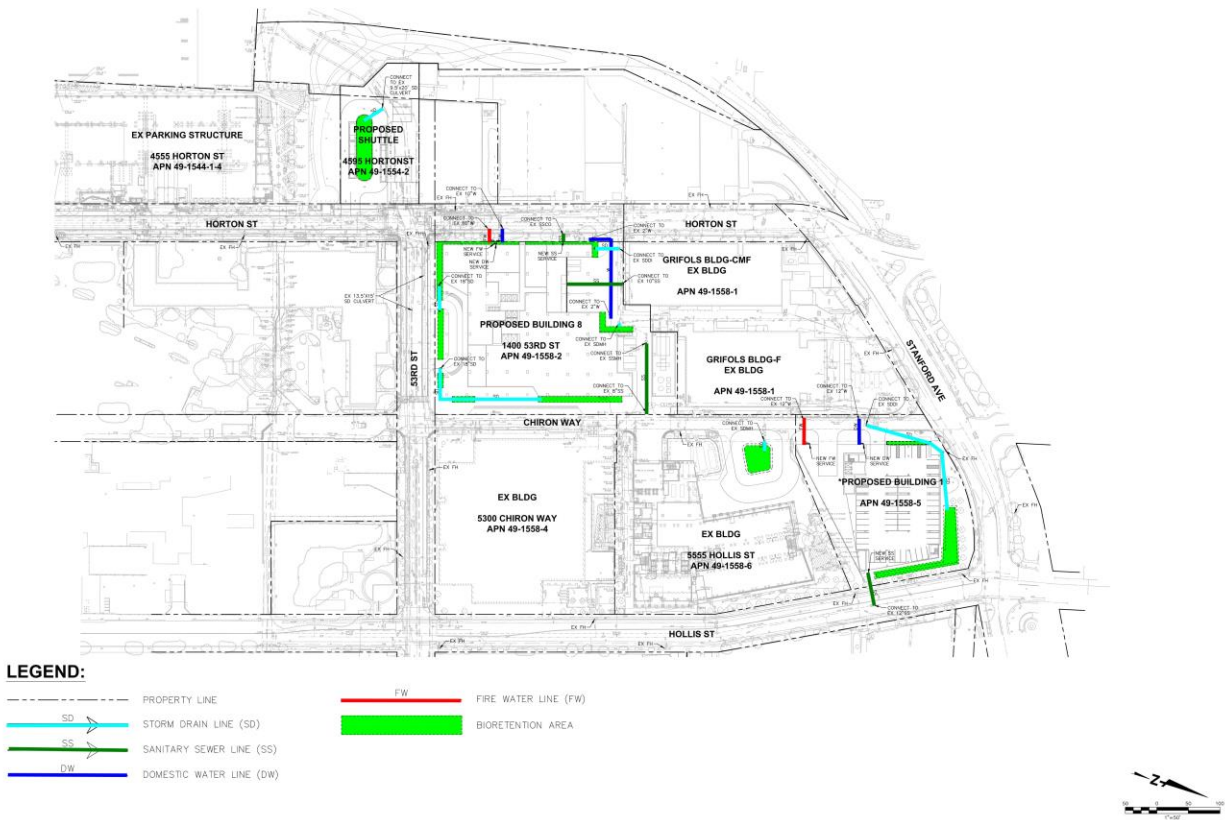


FIGURE 16: CONCEPTUAL UTILITY PLAN

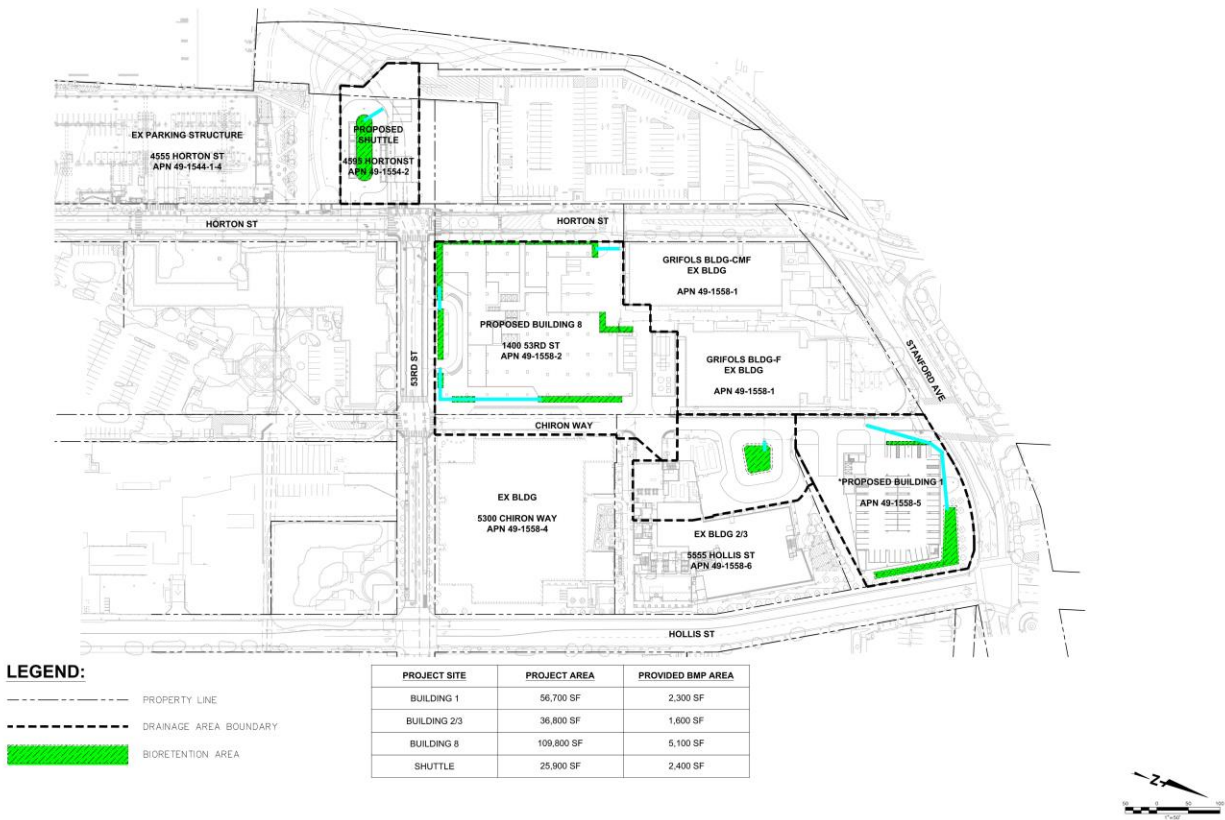


FIGURE 17: CONCEPTUAL ON-SITE STORMWATER MANAGEMENT PLAN

E. Environmentally Protective Enhancements.

In addition to implementing the relevant mitigation measures required by conditions of approval of the PDP, the EMC Project would comply with current laws regarding environmentally protective measures. It would implement the current recommendations of the California Department of Fish and Wildlife to protect nesting birds and roosting bats by conducting pre-construction surveys and comply with the City's Bird Safe Buildings regulations (Muni. Code §§ 9-4.801 to 9-4.804). The EMC Project includes compliance with the City's Urban Forestry Ordinance (Muni. Code §§ 7-10.01 to 7-10.12). The project would implement the energy and water efficiency requirements of current building, energy and landscaping codes, including Cal Green requirements.

The hospital would be designed to meet Title 24 energy code standards and would operate as an all-electric facility. The design would reduce energy demand through the implementation of high-performance building strategies, including advanced mechanical and electrical systems, LED lighting with daylight and occupancy controls, and integrated energy-management technologies. The proposed new Building 8 hospital and Building 1 would utilize all-electric services. Existing Building 2/3 and Building 4 would continue to utilize the same power sources with which they were originally constructed.

F. Estimated Construction Schedule.

Site preparation and construction activities would be coordinated based on the timing of required approvals.

Building 2/3 at 5555 Hollis Street is expected to begin tenant improvement work approximately within 2 months after entitlement approval, with an anticipated construction duration of approximately 21 months. The earliest estimated schedule for the construction phase is Q3 2027 – Q4 2029.

For Building 8, Final Development Plan (FDP) will be submitted towards the end of entitlement process. It is expected to follow a longer construction schedule of about 71 months, and it is anticipated to have some overlap with the construction schedule for Building 2/3. The earliest estimated schedule for the construction phase is Q1 2028 – Q4 2033.

Building 4 at 5300 Chiron Way is planned to proceed with tenant improvements as future program requires. Administrative uses currently occupy a portion of the building. There would be no tenant improvement work done at Building 4 until post occupancy of the hospital. The earliest estimated schedule for the construction phase is Q3 2034 – Q4 2035.

The future Building 1 is estimated to start construction after the hospital is in operation, with an approximately 24 months construction phase. The earliest that construction for Building 1 would begin would be after full occupancy of Building 2/3 (5555 Hollis), Building 4 (5300 Chiron) and Building 8 (Hospital). In the interim, the site would be used as a construction coordination area.